

UPMC Shadyside

5230 Centre Ave, Pittsburgh, PA 15232

DISCHARGE SUMMARY Document #: DIS-002

Patient & Admission

Name	Elizabeth Harvey
Date of Birth	1975-01-25
Phone	724-555-9763
Address	7738 Leon Underpass Apt. 148 Pittsburgh, PA, 15222
MRN	MRN-433200
Admit Date	2026-04-19
Discharge Date	2026-04-22
Attending	Douglas Ford, MD
Attending NPI	4026542355

Insurance

Primary Payer	Highmark BCBS PA
Primary Member #	JIM514846564

Primary Discharge Diagnosis

Type II NSTEMI, demand-related

ICD-10: I21.09, I25.10

Hospital Course

The patient presented to the emergency department with acute onset substernal chest pain and dyspnea. Initial electrocardiogram demonstrated ST-segment depression in the anterior leads with reciprocal changes. Troponin I was elevated at 2.8 ng/mL, and subsequent serial measurements peaked at 4.1 ng/mL, consistent with acute myocardial infarction. The patient was immediately initiated on dual antiplatelet therapy with aspirin and ticagrelor, along with unfractionated heparin. Urgent cardiac catheterization revealed significant stenosis of the left anterior descending coronary artery with TIMI II flow. Given the demand-related nature of the infarction and the patient's hemodynamic stability, percutaneous coronary intervention was performed with successful placement of a drug-eluting stent to the LAD with restoration of TIMI III flow. Post-procedure angiography demonstrated excellent stent apposition with no residual stenosis.

During the hospital course, the patient remained hemodynamically stable with no recurrent chest pain or arrhythmias. Transthoracic echocardiography on hospital day two revealed mild hypokinesis of the anterior wall with preserved overall left ventricular ejection fraction of 48 percent. B-type natriuretic peptide was mildly elevated at 287 pg/mL. The patient was transitioned to oral beta-blocker and ACE inhibitor therapy, which were titrated to target doses. High-intensity statin therapy was initiated, and the patient demonstrated good tolerance to all medications without adverse effects.

The patient was discharged on hospital day three in stable condition with appropriate follow-up arranged. Discharge medications include aspirin, ticagrelor, metoprolol, lisinopril, and atorvastatin. The patient was counseled on medication adherence, cardiac risk factor modification, and the importance of outpatient cardiology follow-up within two weeks for repeat echocardiography and functional assessment.

Procedures Performed

Coronary angiography	CPT 93458
-----------------------------	-----------

Medication Changes

Metoprolol succinate 50 mg daily	STARTED
Aspirin 81 mg daily	STARTED
Ticagrelor 90 mg BID	STARTED

Follow-up Instructions

Recommended Specialist	Cardiology
Follow-up Window	10 days
Urgency Tier	HIGH
Clinical Flags	EF<40%, recent-revascularization

Dictated and electronically signed by:
Douglas Ford, MD
NPI: 4026542355
UPMC Shadyside